



HIV is becoming a chronic condition that needs prolonged medical individual-centered care. People living with HIV have a high burden of *physical, psychological, and social difficulties* which require palliative care that is *holistic* and is recognized by the World Health Organization (WHO) as an essential element of HIV care, from diagnosis to end of life. According to Huang, in the management program of HIV, palliative care should be integral to alleviate the distress and helps the people living with HIV to lead better quality of life. This means, in the continuum of HIV/AIDS, people living with this condition *suffer from symptoms that are caused by opportunistic infections, the virus itself, or the side effects of antiretroviral drugs*. Thus, to improve the individuals' quality of life requires to focus on palliative care, and enforcement of its implementation is critically important.

More importantly, people living with HIV need palliative care for many reasons such as pain, cough, nausea, weakness, fever, diarrhea, breathing problems, other comorbidities, aging, and

complex decision-making. Palliative care for these populations is a means of managing these challenging experiences and minimizing burden on hospital resources. Therefore, in the era of HIV, compressive package of care is critically important and should be in place in all HIV treatment guidelines.

Through caregiving, it is critically important for people living with HIV to address multiple problems related to the diseases, *its medication*, and *complications like pain, drug adverse effects, emotional distress*, and *physical symptoms*. These groups of patients experience a *high degree of emotional problems like anxiety, depression*, and *mental health problems* compared to general population.

Moreover, it is important for people living with HIV because of the following dimensions:

- **Complex disease process:**

People living with HIV are suffering from ranges of several complications and opportunistic infections and related treatment side effects.

- **Complex treatments:**

People living with HIV need lifetime treatment for viral suppression and treatment for comorbidities which will also create stress on the daily life of the patient.

- **Stigmatization and discrimination:**

In many parts of the world, people living with HIV are still suffering from stigmatization and discrimination.

- **Complex family issues:**

The disease has a great impact on the productivity of the family. The financial burden will increase as parents got infected and progress to advance illness.

- **The burden on healthcare workers:**

The healthcare workers face stress related to lack of resources, symptom management, and management of related complications.

Symptom Management- ARV Therapy Side Effects

Antiretroviral therapy (ART) is recommended for all HIV infected individuals. The goal of ART is to inhibit viral replication, decreasing viral load and allowing the patient's immune system to recover. ART also decreases the risk and/or slows the development of AIDS-defining conditions and non-AIDS-defining complications, lessens the risk of HIV transmission, and prolongs life.

Modern ART generally consists of two or more drugs of different classes in combination to prevent the acquisition of drug resistance. Many combinations are available in a single pill, which decreases the pill burden. Adherence is important because no adherence can lead to viral replication in the presence of ineffective levels of one or more of the drugs used in the regimen, causing the virus to mutate and reducing susceptibility to current drugs used to treat HIV.

Prophylactic antimicrobial therapy is administered to certain HIV-infected patients with the goal of preventing a clinically significant opportunistic infection. Such treatment is typically initiated as the individual's cell-mediated immunity (i.e., CD4 cell count) declines.

The treatment of people living with HIV requires the balance between acute and control of symptoms. Many people living with the disease suffer from different symptoms including pain. The common symptoms that require treatment are ***pain, nausea, vomiting, diarrhea, weight loss, sleep disturbance, depression, mouth sore, sadness, fever, cough, skin problem, and respiratory problems.***

The central focus of palliative care is symptom management. For symptom management, ***opioids*** are essential for pain management. It is vitally important to consider the WHO pain management ladder. However, the plan for pain management depends on the individual preference. Managing symptoms enables people living with HIV to function fully. The need for palliative care for people living with HIV arises from the disease and its associated complications like varieties of symptoms and side effects of the drug management.

Pain is the most common symptom patients are suffering from and for which they should be treated if they complain of it. It can be managed both ***medically and psychologically***. Spiritual

and emotional support and counselling should always include the medical management of pain of people living with HIV. Deep breathing techniques, relaxation, and distracting attention of patients will also help as relieving methods for pain symptoms.

Nausea and vomiting are another common distressing symptom the patients need to get advice on foods and feeding habits. The patients need to *drink water, tea, ginger*, etc. to get rid of the symptoms. The person with these problems should identify and avoid aggravating factors.

Diarrhea is also a common problem which needs management. It may be a side effect of antiretroviral regimens; other causes include Cytomegalovirus colitis, cryptosporidiosis, microsporidiosis, giardiasis, Kaposi sarcoma, and other infective agents. *Oral rehydrating solutions should be given to prevent dehydration.* Use oral rehydration solution if there is a large volume of diarrhea. Encourage the patient to drink plenty of fluids to replace lost water. If it is not possible to get oral rehydrating solutions, local remedies could be considered. To prevent and control diarrhea symptoms, it is advisable to avoid eating raw foods. In general, for diarrhea disease management, it is recommended to increase frequency of food intake, such as rice soup, porridge, bananas, and other soups.

Mouth sore is perhaps more than any other symptoms that could affect communication as well as comfort of people living with HIV. A sore mouth is very common in palliative care and may be severe in people living with HIV. It causes painful swallowing which may be caused by Cytomegalovirus ulcers of the mouth or esophagus, virus, or fungal infection. In most cases it can be prevented by keeping the mouth clean and moist and treating any infections promptly. Nystatin oral drops after food and at night hold dose in the mouth to allow it to act topically. For fluconazole 50 mg daily for 5 days, use higher doses (200 mg daily for 2 weeks) if patient has difficulty swallowing and you suspect esophageal candidiasis. Rinsing the mouth with diluted salt water after eating and at bedtime is the recommended care for patients with mouth sore.

Fever in people living with HIV infection usually suggests an infectious complication. It may be a side effect of antiretroviral regimens; if suspected, assess and treat cause with paracetamol or acetylsalicylic acid, and encourage the patient to drink water, diluted tea, or fruit juice

frequently. Fever, cough, weight loss, and night sweats in an HIV-infected person may indicate a heightened suspicion of tuberculosis (TB). At all stages of HIV, TB must be diagnosed, treated, and controlled. If fever is persisting, palliative care team must agree to do all that could minimize suffering.

Weight loss is also another challenge in the management of HIV conditions despite the use of antiretroviral therapy. Resting energy expenditure is high, and, therefore, food intake should be increased to prevent detectable risk of death. There are different related factors with weight loss in people living with HIV. These include mouth sore, disease of the upper gastrointestinal tract, anorexia, side effects of drugs, food insecurity, malabsorption due to diarrheal disease, and poorly controlled infections.

Sleep disturbance like insomnias, which is a subjective complaint of inadequate nocturnal sleep, is manifested as difficulty of initiating or maintaining sleep, early-morning awakening, non-restful sleep, or a combination of all of these. In many cases insomnia is a symptom of another mental or physical disorder which includes pain, anxiety, depression, drug withdrawal, and side effects of certain ARV regimens, especially those with efavirenz. A general care for sleep disturbance includes:

- Assessing and managing the cause.
- Reducing the intake of stimulants and avoiding alcohol near bedtime.
- Exercising regularly in the earlier part of the day.
- Providing benzodiazepines.

Respiratory symptoms are also common problems people living with HIV are suffering from. For instance, cough or difficulty breathing may be due to common opportunistic infections seen in people living with HIV. Difficulty in breathing is a frightening experience due to fear of death. Therefore, as a general care:

- Position the patient to sitting to maximize ventilation.
- Ensure good ventilation by opening windows, using a fan.
- Adjust activity to help slow and deep breathing.

For a Patient with HIV/AIDS

Caregivers should take care to avoid infection. There is an extremely low risk of contracting HIV/AIDS by caring for someone who is infected. However, it is important that the following precautions are taken:

- HIV is present in blood and body fluids—wear gloves when contacting these fluids. Keep wounds covered (both those of the caregiver and the person with HIV/ AIDS).
- There is no risk from casual household contact (no gloves needed).
 - clean up blood, faeces, urine with ordinary household bleach.
 - clean cutlery, linen, bath, etc. with ordinary washing products.
- Keep clothing and sheets stained with blood, diarrhea or other body fluids separate from other household laundry. Use a piece of plastic, or paper, gloves or a large leaf to handle soiled items.
- Do not share toothbrushes, razors, needles, or other sharp instruments that pierce the skin.
- Wash hands with soap and water after changing soiled bed sheets and clothing and after any contact with body fluids.
- Use condoms if sexual activity.
- Bathe patient without gloves if neither caregiver or patient has wounds.

